

ENVIRONMENTAL HEALTH

Environmental health refers to the branch of public/community health that focuses on understanding how the environment impacts human health and well-being. It encompasses the study and management of various environmental factors

Environmental sanitation refers to the collective efforts and measures undertaken to maintain and promote cleanliness, hygiene, and health within the environment to safeguard human well-being. It encompasses a wide range of practices and interventions aimed at preventing the spread of diseases, reducing environmental pollution, and ensuring the sustainable management of natural resources. Key components of environmental sanitation include:

Goals of Environmental Health

The goal of environmental health is to:

- identify and mitigate environmental hazards,
- prevent disease and injury,
- promote health equity, and
- create environments that support human health and well-being.

Environmental health is an interdisciplinary field that draws upon principles from epidemiology, toxicology, environmental science, sociology, urban planning, and other disciplines to address complex interactions between people and their surroundings.

By assessing and managing environmental risks, environmental health professionals strive to protect public health and create healthier communities for current and future generations.

Components of Environmental Health

Environmental health encompasses a broad range of components, including the physical, built, chemical, biological, and social environments. Recognizing the interconnectedness of these factors is essential for addressing environmental health challenges and promoting health equity. By addressing environmental risks and promoting sustainable practices, we can create healthier communities and safeguard the well-being of current and future generations. The components of environmental health and their impact on human health are as follows:

Physical Environment

Air – The composition of the air we breathe, including pollutants such as particulate matter, ozone, and nitrogen dioxide, can affect respiratory health and contribute to conditions like asthma and cardiovascular disease.

Water – Access to clean and safe water is essential for preventing waterborne diseases. Contaminants such as bacteria, viruses, heavy metals, and chemicals can pose health risks if present in drinking water.

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Soil – Soil contamination with pollutants like heavy metals, pesticides, and industrial chemicals can affect food safety and agricultural productivity. Exposure to contaminated soil can lead to health issues such as lead poisoning and cancer.

Built Environment

Housing – Housing conditions affect indoor air quality, exposure to environmental allergens, and safety hazards such as lead paint and asbestos. Adequate housing standards and housing rehabilitation programs are critical for protecting residents' health.

Urban Design – Urban planning decisions influence transportation patterns, access to green spaces, and exposure to environmental pollutants. Sustainable urban design strategies can promote physical activity, reduce pollution, and enhance the overall health and well-being of its people.

Chemical Environmental

Toxic Substances – Chemicals in consumer products, industrial processes, and pollution sources can have adverse health effects. Examples include pesticides, heavy metals, industrial chemicals, and endocrine-disrupting compounds.

Hazardous Waste – Improper disposal of hazardous waste can contaminate soil, water, and air, posing risks to human health and the environment. Hazardous waste management is critical for preventing pollution and reducing exposure to harmful substances.

Biological Environment

Infectious Organisms – Microorganisms such as bacteria, viruses, fungi, and parasites can cause infectious diseases. Factors influencing the spread of infectious diseases include population density, sanitation practices, and vector control measures.

Zoonotic Diseases – Diseases transmitted between animals and humans, such as rabies, Ebola, and Lyme disease, highlight the interconnectedness of human and animal health. Zoonotic disease surveillance and control efforts are essential for preventing outbreaks.

Social Environment

Socioeconomic Factors – Social determinants of health, including income, education, employment, and access to healthcare, influence environmental exposures and health outcomes. Disparities in environmental health often mirror broader socioeconomic inequalities.

Community Characteristics – Neighborhood conditions, such as housing quality, urban infrastructure, and green spaces, can impact residents' exposure to environmental hazards and their overall well-being. Community-based interventions are essential for addressing environmental justice issues.

ROLES AND RESPONSIBILITIES OF HEALTHCARE TEAM MEMBERS

In community health nursing, collaboration among various healthcare team members is crucial for providing comprehensive care to individuals, families, and communities. While nurses often play a central role in community health, they work closely with other healthcare professionals to address a wide range of health needs. By working together as a cohesive team, healthcare professionals in

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community health nursing can address the complex health needs of individuals and communities, promote health equity, and improve population health outcomes.

Here are the roles and responsibilities of some key healthcare team members in community health nursing:

Physicians

Role: Physicians provide medical expertise and guidance in diagnosing and treating health conditions in the community.

Responsibilities:

- Collaborate with community health nurses in assessing community health needs and developing health promotion and disease prevention programs.
- Provide medical oversight and consultation for complex cases and referrals.
- Participate in interdisciplinary team meetings to discuss patient care plans and strategies for improving community health outcomes.

Public Health Educators

Role: Public health educators design and implement health education programs to promote health literacy and behaviour change in the community.

Responsibilities:

- Develop educational materials and resources on topics such as nutrition, exercise, chronic disease management, and preventive health measures.
- Conduct community health education workshops, classes, and outreach events to raise awareness and empower individuals to make informed health decisions.
- Collaborate with community health nurses to assess educational needs, identify priority health issues, and tailor education programs to meet the needs of diverse populations.

Social Workers

Role: Social workers address social determinants of health and provide support services to individuals and families in the community.

Responsibilities:

- Conduct psychosocial assessments to identify social, economic, and environmental factors influencing health outcomes.
- Assist individuals and families in accessing community resources, such as housing assistance, financial aid, transportation services, and mental health support.
- Collaborate with community health nurses to develop care plans that address social and emotional needs, promote self-sufficiency, and enhance overall well-being.

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- Providing counselling and support to patients and families facing medical crises or chronic illnesses
- Advocating for patients' rights and collaborating with interdisciplinary teams to address complex social issues impacting healthcare

Nutritionists/Dietitians

Role: Nutritionists and dietitians promote healthy eating habits and provide dietary guidance to individuals and communities.

Responsibilities:

- Conduct nutrition assessments and develop personalized dietary plans for individuals with specific health conditions or dietary needs.
- Educate community members about the importance of nutrition, healthy eating habits, and disease prevention through nutrition education programs and counselling sessions.
- Collaborate with community health nurses to address nutritional deficiencies, promote healthy lifestyle choices, and prevent diet-related health problems in the community.

Community Health Workers (CHWs)

Role: Community health workers serve as liaisons between healthcare providers and community members, providing culturally competent care and support.

Responsibilities:

Conduct outreach and health screenings in the community to identify individuals at risk for health problems and connect them with appropriate resources.

Provide health education and support to community members on topics such as preventive care, chronic disease management, medication adherence, and lifestyle modifications.

Collaborate with community health nurses to facilitate access to healthcare services, promote health equity, and address health disparities in underserved communities.

District Health Management Team (DHMT)

Role: The DHMT oversees the coordination and delivery of healthcare services in the community.

Responsibilities:

- Develop and implement policies, procedures, and protocols for community health programs and services.
- Allocate resources, manage budgets, and ensure compliance with regulatory requirements and quality standards.

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- Collaborate with community health nurses and other team members to evaluate program effectiveness, identify areas for improvement, and implement quality improvement initiatives to enhance patient outcomes and satisfaction.

Allied Health Professionals (e.g., physical, occupational, and speech therapists)

Role: Allied health professionals specialize in specific therapeutic interventions to improve patients' physical, mental, and emotional well-being.

Responsibilities:

- Conducting assessments to identify patients' needs and developing personalized treatment plans.
- Providing therapeutic interventions to improve mobility, function, and independence.
- Educating patients and caregivers on therapeutic exercises and techniques.
- Collaborating with other healthcare team members to coordinate care and achieve treatment goals.

By understanding and appreciating the roles and responsibilities of fellow team members, healthcare professionals can work together synergistically to achieve optimal outcomes for patients.

COMMUNITY MOBILIZATION

Definition

Community mobilization (CM) refers to the process of inspiring, organizing, empowering, and motivating community members to address health issues, improve quality of life, and achieve common goals.

Community mobilization is a vital strategy in public health aimed at empowering communities to actively participate in identifying and addressing health issues affecting their well-being. It involves engaging community members, building partnerships, and mobilizing resources to create sustainable solutions to health challenges.

Community mobilization is a powerful tool for promoting health, fostering community empowerment, and addressing health inequities. By engaging and empowering communities to take the necessary action, we can create sustainable solutions to health challenges, build stronger and more resilient communities, and improve the well-being of populations around the world.

Principles of CM

Empowerment: Empowering individuals and communities to take control of their health and well-being.

Participation: Encouraging active participation and involvement of community members in decision-making processes.

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Collaboration: Fostering partnerships and collaboration among diverse stakeholders, including community members, organizations, and institutions.

Sustainability: Promoting long-term solutions and initiatives that are sustainable and adaptable to changing community need

Importance of CM

- Builds a sense of ownership and responsibility among community members for addressing health issues.
- Facilitates collaboration and partnerships between communities, healthcare providers, and other stakeholders.
- Empower communities to take ownership of their health and drive positive change.
- Ensures that health interventions are culturally relevant, responsive to community needs, and respectful of local customs and traditions.
- Motivate communities to advocate for policy changes to respond better to their health needs
- Promotes sustainable solutions by leveraging local resources, capacities, and strengths within the community.

Strategies for CM

Community Engagement: Engage community members through meetings, focus groups, surveys, and other participatory methods to identify health priorities and develop action plans.

Capacity Building: Build the capacity of community members through training, skill-building workshops, and education programs to enable them to lead and implement health initiatives.

Advocacy: Advocate for policy changes, resources, and support from government agencies, policymakers, and other influential stakeholders to address community health needs.

Social Marketing: Use social marketing techniques to raise awareness, change attitudes, and promote healthy behaviours within the community.

Networking and Collaboration: Establish partnerships and networks with community-based organizations, healthcare providers, schools, businesses, and other stakeholders to leverage resources and expertise.

Note: While the importance of community mobilization highlights its significance for empowering communities, promoting health equity, and driving positive change, the strategy of community mobilization outlines specific approaches and techniques for engaging community members, building partnerships, and fostering collective action toward common goals. Both aspects are essential for effective community mobilization and advancing public health objectives.

Steps in Community Mobilization

- Assess community assets, needs, and resources to identify priority health issues and opportunities for intervention.
- Develop a comprehensive community mobilization plan that outlines goals, objectives, strategies, and timelines for action.

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- Identify and mobilize key stakeholders within the community to implement the set plan
- Implement planned activities, engage community members, and mobilize resources to achieve desired outcomes.
- Evaluate the effectiveness and impact of community mobilization efforts
- Collect feedback, and make necessary adjustments as needed to improve outcomes.
- Develop strategies for sustaining community mobilization efforts over time, including building local leadership, securing funding, and institutionalizing initiatives within existing systems.



Diagrammatic Illustration of CM

Community Participation

Refers to the active involvement of individuals, groups, and organizations within a community in decision-making processes, problem-solving, and action planning to address shared concerns, improve well-being, and promote positive change.

It encompasses a range of activities, from volunteering and advocacy to collaborative partnerships and grassroots initiatives, aimed at empowering community members to contribute their knowledge, skills, and resources toward common goals.

Community participation is rooted in the principles of democracy, equity, and social justice, emphasizing inclusivity, transparency, and respect for diverse perspectives. By engaging community members as partners in the planning, implementation, and evaluation of initiatives, community participation fosters ownership, sustainability, and collective action toward creating healthier, more resilient communities.

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COMMUNITY DIAGNOSIS

Definition:

Community diagnosis (CD) is a process of assessing the health status, determinants, and resources within a defined community to identify health priorities and develop strategies for intervention. It provides a systematic approach to understanding the health needs, assets, and challenges within a community.

It involves the collection, analysis, and interpretation of data to identify priority health issues and develop targeted interventions.

It involves analyzing various factors, including demographic characteristics, health indicators, social determinants of health, environmental factors, and community assets.

Community diagnosis is a fundamental process in public health practice, providing a systematic approach to understanding and addressing health needs within a community. By engaging in community diagnosis, public health professionals can identify priority health issues, develop targeted interventions, and work collaboratively with communities to improve health outcomes and promote well-being.

Types of CD

There are two main types of community diagnosis:

- Comprehensive community diagnosis – aims to obtain general information about the community
- Problem-oriented community diagnosis – responds to a particular need

Importance of CD

- Helps in identifying priority health issues and unmet needs within the community.
- Provides data-driven insights to inform policy development, program planning, and resource allocation.
- Guides the development and implementation of targeted interventions tailored to the specific needs of the community.
- Serves as a basis for monitoring progress, evaluating the impact of interventions, and adapting strategies as needed.

REMEMBER! At the stage of identifying problems, it is not good to say: 'I know what your problems are.' It is essential to encourage the community to identify their own problems first; then they will be more ready to deal with them. Secondly, get them to participate in finding solutions. Communities have different amounts of resources, and they also have different values and beliefs. Things that work best for one person or one community may not work for another. So do not assume you know what the best solution is.

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Preparation Steps in CD

- Involve the community people in planning – they have a great amount of experience and insight into what works for them, what does not work for them, and why. Involving local people can increase their commitment to the programme and it can help them to develop appropriate skills and knowledge to identify and solve their problems on their own. They may help to increase the resources available for the programme, promote self-help and self-reliance, and improve trust and partnership between the community and health workers. It is also a way to bring about 'social learning' for both health workers and local people. Therefore, if you involve the local community in a programme which is developed for them, you will find they will gain from these benefits.
- Community diagnosis is done using a tool called "Health Indicators" which are variables used for the assessment of community health. The health indicators must be – valid, reliable, sensitive, specific, feasible, and relevant. These health indicators can be classified as mortality indicators, morbidity indicators, disability rates, nutritional status indicators, health care delivery indicators, utilization rates, indicators of social and mental health, environmental indicators, socioeconomic indicators, health policy indicators, indicators of quality of life and other indicators.
- Identify the community boundaries and resources – natural and artificial. Natural resources include labour force (human beings – such as the number of men, women and children), plots of land, rivers or lakes, and animals. Artificial resources include infrastructures such as roads, health facilities, markets schools, etc., and means of transportation.
- Review existing health data. E.g., how many numbers of malaria, TB, Leprosy etc.
- Identify the community's cultural activities. Culture is a complex way of living. Language, social and economic activities, and mode of dressing. Dietary habits and religion are the components of culture. The health of the community may be adversely affected by their cultural activities.

Process of CD

- Gather data from multiple sources, including surveys, health records, community assessments, interviews, and secondary data sources.
- Analyze collected data to identify patterns, trends, disparities, and areas of concern related to health outcomes and determinants.
- Interpret findings in the context of community characteristics, social determinants, cultural factors, and health disparities.
- Engage community members, stakeholders, and partners throughout the diagnosis process to ensure a participatory approach and promote community ownership.
- Prioritize health issues based on their significance, prevalence, severity, and impact on the community's health.
- Develop action plans outlining goals, objectives, strategies, and timelines for addressing priority health issues.
- Implement planned interventions, mobilize resources, and collaborate with partners to address identified health needs.

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- Monitor progress, evaluate outcomes, and reassess the community's health status to track improvements and identify areas for further action.

Tools and Methods for Community Diagnosis

Community Health Assessment Surveys: Surveys conducted to collect information on health behaviours, risk factors, and access to healthcare services within the community.

Health Indicators and Data Sources: Use of health indicators and data from sources such as vital statistics, health records, disease surveillance systems, and census data.

Community Asset Mapping: Identifying community strengths, resources, and assets that can be leveraged to address health challenges.

Qualitative Research Methods: Conducting focus groups, interviews, and community forums to gather insights into community perceptions, needs, and priorities.

Geographic Information Systems (GIS): Mapping and spatial analysis of health data to identify geographic patterns and disparities in health outcomes

Characteristics of Community Diagnosis

- ❖ Ability to address important community problems
- ❖ Ability to identify most of the targeted health events
- ❖ Adequacy in reflecting changes in the distribution of events over time, place and person
- ❖ Participatory, uncomplicated, sensitive, timely and inexpensive

Purpose of Community Diagnosis

- ❖ To identify and quantify health problems
- ❖ To identify people at risk
- ❖ To identify community needs and problems
- ❖ To determine available resources
- ❖ To set priorities for planning

REFERRAL SYSTEM

Definitions:

It is a systematic problem-solving approach involving a series of actions that help clients to utilize resources for the purposes of resolving needs.

It is a set of processes of systematically referring a patient from a health care provider to another health facility to address his/her needs and know the outcome of the referral.

The referral system is a system that provides for a patient or client whose care is transferred

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from one health facility to another for certain reasons.

Goal

The health care provider directs the client to a service or resource, with the goal of promoting high-level wellness and enhancing self-care capabilities and quality of care.

In all healthcare systems, there is always a reason to refer a patient/client from one facility to another. The health care systems of Sierra Leone operate at three levels; primary, secondary, and tertiary levels. Referral could be from primary to secondary and tertiary or from tertiary to secondary and primary making it a two-way system.

Types of Referrals

Internal referral – a system of referral within a hospital.

External referral – a process of referral from one health facility to another health facility or institution.

Referral Policies

1. Patients shall have the right:
 - a) To know the reason(s) for referral, and
 - b) Participate in the choice of facilities where he/she is referred
2. Health care providers have the responsibility to ensure:
 - a) Prompt and appropriate response to patients' health needs by immediate referral of services.
3. A two-way functional referral must be observed by:
 - a) Ensuring that the receiving facility provides feedback to the referring facility (referral feedback is a process by which a receiving facility informs the referring health facility of the referral outcome)
4. It is a shared responsibility of referring and receiving facilities to exert all efforts to ensure that referred patients are not lost during the referral process.
5. All referring facilities/providers must use a standard referral form.
6. All hospitals shall maintain a referral logbook.
7. Patients who are not referred under policies and procedures shall be accommodated and evaluated accordingly.

Reasons for Referral

- ❖ Referral for specialized care to a tertiary health facility: e.g., Cases of fractures are referred from primary to secondary health facility or even tertiary facility if there are complications. Cases of pregnancy-induced hypertension or other obstetric life-threatening complications are referred from primary through secondary to tertiary obstetric hospitals.
- ❖ Patient with doubtful medical diagnosis: Because primary or secondary facilities may take the appropriate diagnostic equipment, they may not be able to establish a definitive medical diagnosis of certain clients such patients are referred for further clinical assessment.
- ❖ Patients who have been diagnosed and treatment commenced but who have not been responding to treatment are expected to be referred for further clinical assessment to establish the appropriate changes.

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- ❖ Patients with life-threatening conditions must be referred directly to tertiary facility with a critical care unit for appropriate management.
- ❖ Some patients even though they can be managed at primary level may still request for management at a tertiary level. E.g., client with no complicated pregnancy request to deliver at the obstetric hospital even though that could have been done at the peripheral health unit. The decision of the client has to be respected especially where it will not result to any harm.
- ❖ Patients who have been successfully managed at tertiary or secondary levels are referred back to the peripheral health unit in the community for follow up management. E.g., a patient who had a prostatectomy operation at a tertiary hospital who recovers completely is referred for follow up care at his community level.

Steps to Observe in Referrals

- ❖ Establish a working relationship with the client and explain the need for the referral
- ❖ Set objectives for the referral and explore resource availability at the receiving facility
- ❖ It is always necessary for an initial health assessment to be done and emergency intervention implemented prior to referral
- ❖ Referral starts with informing the client and significant others about the need for referral, what is their role in the process, and what is involved in terms of needed resources.
- ❖ Transport arrangement is made and sometimes it may be necessary for a nurse to accompany the patient depending on the patient's condition.
- ❖ The patients or significant others must consent and remember that the client can choose whether or not to adhere to the referral
- ❖ The health worker has the responsibility to allay fear in the client against the challenges of relocation to the next health facility.
- ❖ Writing the referral note. The client must be referred with a referral note. This note must be written by the health worker who is directly in charge of the management of the client. Sometimes there is a standard referral form in the health system. This form must be completely filled in and signed by the person referring the client. The referral note must have the following information;
 - The health facility from which the client is referred
 - The client's personal information: Name, Sex, Age, Occupation, Marital status, Address, etc.
 - The clients complain for which consultation was made
 - Initial assessment at the health facility and finding
 - Emergency intervention prior to referral
 - Reason for referral
 - Health facility to which the client is referred

- Name, Signature, and designation of the health worker referring, referral time, and date
- ❖ Evaluate and follow-up

Challenges of Referral

The referral system could be faced with several challenges which may endanger the client's health. Some of these challenges are as follows;

- **Delay in referral** – sometimes there is a delay in the referral of a client as a result of ignorance of the health workers' non-compliance to their units or continuous demand for money from the client or significant others.
- **Cultural barriers** – some clients may not consent to referral because of cultural barriers, changing environment, language, or fear of interference into their privacy or beliefs
- **Transportation problems** – normally transporting a client from one health facility to another is through an ambulance. This may not be available.
- **Difficult terrain** – sometimes the terrain may not be motorable which will hinder the transportation of the client.
- **Poverty** – plays a very important role on attendance at the health facility. Client may rather look for alternative traditional treatment and refuse referral because this will have to involve more money which they cannot afford.

Summary: The referral should be merited, practical, tailored to the client, up to the client to say 'yes' or 'no', timely, coordinated with other activities, and incorporated, so the client has input into the planning and implementation process. **Also, remember that a referral without follow-up is useless.**

HEALTH EDUCATION

Definitions:

Health education is the dissemination of knowledge, information, and skills to individuals, families, and communities to promote health, prevent diseases, and enhance well-being.

Health education and communication are integral components of primary healthcare, aiming to promote health literacy, empower individuals, and facilitate behaviour change.

Definitions by different Authors and Experts

Health education has been defined in many ways by different authors and experts. Some of these definitions include but are not limited to the following:

1. Health education is the sum of the experiences, which favourably influence habits, attitudes and knowledge relating to the individual, community and social health. (**Thomas Wood**)
2. Health education is like general education which is concerned with changes in the knowledge, feelings and behaviour of people. In its most usual form, it concentrates on developing such health practices as are believed to bring about the best possible state of well-being. [**W.H.O Technical Report** (1954)]
3. Health education is "a combination of learning experiences designed to facilitate voluntary actions conducive to health."

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The terms combination, designed, facilitate and voluntary action have significant implications in this definition. **Combination** – emphasizes the importance of matching the multiple determinants of behaviour with multiple learning experiences or educational interventions. **Designed** - distinguishes health education from incidental learning experiences as systematically planned activities. **Facilitate** – means create favourable conditions for action. **Voluntary action** means behavioural measures are undertaken by an individual, group or community to achieve an intended health effect without the use of force, i.e., with full understanding and acceptance of purposes. (**Lawrence Green**)

Aims of Health Education

- To promote the health of the mind and the body
- To promote the use of preventive health services
- To enable people to change their attitudes and behaviour and adopt better health habits. That is, set an example of desirable health behaviour
- To assist individuals and communities to identify health problems, take responsibility for their health, and understand their role in health and medical agencies in meeting those problems.
- To enable people to develop a scientific point of view of health about the traditional and modern concepts of health.
- To enable individuals to take interest in current events related to health
- To enable the people to arrive at suitable conclusions based on scientific knowledge and act as individual members of the family and community for protecting, maintaining, and promoting individual, and community health.

Communication and Health Education Activity Terminologies

Information, Education and Communication (IEC) is a term originally from family planning and more recently HIV/AIDS control programme in developing countries. It is increasingly being used as a general term for communication activities to promote health.

- **Information:** A collection of useful briefs or detailed ideas, processes, data, and theories that can be used for a certain period of time.
- **Education:** A complex and planned learning experience that aims to bring about changes in cognitive (knowledge), affective (attitude, belief, value), and psychomotor (skill) domains of behaviour.
- **Communication:** the process of sharing ideas, information, knowledge, and experience among people using different channels.

Social Mobilization: is a term used to describe a campaign approach combining mass media and working with community groups and organizations.

Health Extension: is an approach of promoting change through demonstration, working with opinion leaders and community based educational activities.

Nutrition Education: is education directed at the promotion of nutrition and covers choice of food, food preparation and storage of food.

Family Life Education: refers to education of young people in a range of topics that include family planning, child rearing and childcare and responsible parenthood.

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Patient Education: is a term for education in hospital and clinic settings linked to following treatment procedures, medication, and home care and rehabilitation procedures.

Behaviour Change Communication (BCC): Is an interactive process aimed at changing individual and social behaviour, using targeted, specific messages and different communication approaches, which are linked to services for effective outcomes.

Advocacy: refers to communication strategies focusing on policy makers, community leaders and opinion leaders to gain commitment and support. It is an appeal for a higher-level commitment, involvement and participation in fulfilling a set program agenda.

Importance of Health Education

Since health is a precious asset, then education on health is indeed imperative. Many people are ignorant of the causes of illness. They may be afraid of seeking treatment and hide their illnesses or ailments. Prevention is better than cure and it is much cheaper and prevents economic loss of human power, time, suffering, and distress. Health teaching may be very well accepted when given in a curative or clinical environment. Health education is relevant in the following ways:

- It provides information about the functions of the body, the role of health and hygiene, and precautionary measures for keeping off diseases.
- It helps to discover physical defects and various types of abnormalities in children.
- It helps to develop health habits such as the need for fresh air, hygienic feeding, and other healthy habits.
- Health education helps to develop better human relations
- It provides knowledge regarding the prevention and control of various diseases

Basic Principles of Health Education

- All health education should be need-based. Therefore, before involving any individual, group, or community in health education with a particular purpose or for a program the need should be ascertained. It has to be also specific and relevant to the problems and available solutions.
- Health education aims at change of behaviour. Therefore, multidisciplinary approach is necessary for understanding of human behaviour as well as for effective teaching process.
- The health educator must be aware of the customs, health problems, health needs and attitude of the local community. Therefore, Respect must always be shown to the community and for community customs that are not harmful.
- It is necessary to have a free flow of communication. The two-way communication is particularly of importance in health education to help in getting proper feedback and get doubt cleared.
- The health educator should use terms that can be immediately understood. Highly scientific jargon should be avoided.
- It must be remembered that people are not absolutely without any information or ideas. The health educators are not merely passing information but also give an opportunity for the clients to analyse fresh ideas with old ideas, compare with past experience and take decisions that are found favourable and beneficial.

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- Health Education is based on scientific findings and current knowledge. Therefore, a health educator should have recent scientific knowledge to provide health education. viii. Persuasion is more effective. The health educator has to adjust his talk and action to suit the group for whom he has to give health education. E.g., when the health educator has to deal with illiterates and poor people, he has to get down to their level of conversation and human relationships so as to reduce any social distance. The educator should be acceptable to the local community. The health educators have to make themselves acceptable. They should realize that they are enablers and not teachers in order to have to win the confidence of clients.
- The time, place, and people to be taught must be carefully considered as well as the topic and method chosen to meet their needs.
- The most essential and important topics should be the first priority
- The public health nurse should evaluate his/her own teaching to see if it is effective.
- Health Education should provide an opportunity for the clients to go through the stages of identification of problems, planning, implementation and evaluation. Involvement in planning programs and participation by the learners is necessary for good results. This is of special importance in the health education of the community where the identification of problems and planning, implementing and evaluating are to be done with full involvement of the community to make it the community's own program.
- The health educators should not only have correct information with them on all matters that they have to discuss but also should themselves practice what they profess. Otherwise, they will not enjoy credibility.
- The grave danger with health education programs is the pumping of all bulk of information in one exposure or enthusiasm to give all possible information. Since it is essentially a learning process, the process of education should be done step-by-step and with due attention to the different principles of communication.
- Health Education should start from the existing indigenous knowledge and efforts should aim at small changes in a graded fashion and not be too ambitious. People will learn step by step and not everything together. For every change of behaviour, a personal trial is required and therefore health education should provide opportunities for trying out changed practices.

Approaches to Health Education

- The **persuasion approach** – a deliberate attempt to influence the other persons to do what we want them to do (*directive approach*)
- The **informed decision-making approach** – giving people information, problem-solving, and decision-making skills to make decisions but leaving the actual choice to the people. E.g., family planning methods.

Many public health nurses feel that instead of using persuasion it is better to work with communities to develop their problem-solving skills and provide the information to help them make informed choices. However, in situations where there is a serious threat such as an epidemic,

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and the actions needed are clear-cut, it might be considered justified to persuade people to adopt specific behaviour changes.

Targets for Health Education

Individuals – such as clients, patients, healthy individuals

Groups – E.g., groups of students in a class, youth club, pregnant women at the antenatal clinic etc.

Community – E.g., people living in a village or locality.

Health Education Settings

When considering the range of health education interventions, they are usually described in relation to different settings. Settings are used because interventions need to be planned in light of the resources and organizational structures peculiar to each. Thus, health education and promotion take place, amongst other locations, in:

- Healthcare facilities - antenatal clinics, under-fives clinics, hospital wards
- Schools
- Communities
- Worksites
- Prisons
- Refugee camps
- Local organizations (parent, teacher, religious groups, and scouts, at work).
- Individual teaching in home visiting, and personal counseling.
- The mass media: E.g., television, radio, newspapers, and magazines. This is important in reaching large numbers of people in widely scattered areas.

Role of Health Educator

- Talking to the people and listening to their problems
- Thinking of behaviour or action that could cause, cure and prevent health problems.
- Investigating reasons for people's behaviours
- Helping people to see the reasons for their actions and health problems.
- Asking people to give their own ideas for solving the problems.
- Helping people to look at their ideas so that they could see which were the most useful and the simplest to put into practice.
- Encouraging people to choose the idea best suited to their circumstances.

Remember!!

Health education is the duty of everyone engaged in health and community development activities. If health and other workers are not practicing health education in their daily work, they are not doing their job correctly. ***When treating someone with a skin infection or malaria, a health worker should also educate the patient about the cause of the illness and teach preventive skills. Giving drugs alone will not solve the problem.*** Without Health Education, the patient may fall sick again from the same

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disease. Health workers must also remember that their personal example serves to educate others.

District Health System

The Ministry of Health (MoH) is responsible for all policy direction and is organized into two main divisions at the central level: medical services and management services. At the district level, the same two-division approach is adopted; district health services and the district health management both under the leadership of the District Medical Officer (DMO).

The Management Services which is the Administrative wing of the ministry has four units: Financial Resources, Internal Audit, Human Resources and Support Services. These are further subdivided into smaller units overseen by managers. The Directors and Managers under the Chief Medical Officer are responsible for the preparation and implementation of the central level technical and support programme activities.

At the District Level, the District Health Management Team (DHMT) coordinates, oversees and supervises all Public Health Care activities delivered through the Health Care Delivery system. The team is headed by the DMO who reports to the Director of PHC and to the Programme Manager for Child Health/EPI activities at central level. Within each district, there are Peripheral Health Units (PHUs) and a Government District Hospital. The DHMT is responsible for managing the implementation of District Health Plans. With the decentralization of health service delivery, the Local Councils now provide funding and other forms of support for Public Health activities at the district level. There are three types of PHUs based on the size of the catchment population they serve and their location. The highest of them is the Community Health Centre (CHC). The Head of the CHC is the Community Health Officer, supported by Community Health Assistant, State Enrolled Community Health Nurses (SECHN), Maternal and Child Health Aides (MCHA), Vaccinators, Porters, Cleaners and Watchmen. PHUs are directly responsible to the District Health Management Teams. PHUs provide child health/EPI Services at community level. They are the outlets at which vaccines and other child health/EPI supplies are utilized.

The composition of District Health Management Team

The District Medical Officer is the Administrative Head and is responsible for convening regular meetings of the team comprises the following:

- The Medical Officer or Medical Superintendent or Specialist in charge of the district Hospital
- The District Health Sister (DHS)
- The District Environmental Health Superintendent (DPHS);
- The Hospital Matron
- The Monitoring & Evaluation Office
- The Health Education Officer
- The District Pharmacist
- The Health Administrator or Hospital Secretary
- The WASH Co-coordinator.

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- Representative of Community Health Officers
- Finance Officer
- District Operation Officer (D.O.O.),
- District Social Mobilization Officer,
- Birth and Deaths Registrar
- MCH Aides Training Coordinators.
- Disease Surveillance Officer

Other personnel may be co-opted into the team as and when their expertise is required. These include, Specialist Physician, “Head of Health Training schools, or persons from International Agencies or Missions who work in the health field.

The District Health Management Team needs to meet at least once every month and or more often when necessary e.g. when training programmes are underway or at times of emergency, like epidemics.

The District Health Management Team’s Office

1. This office should have a large wall map on which are marked the location and type of all the Peripheral Health Units in the district. This map is used to study the relation of villages to the PHUs, the population coverage and access to health facilities. It is also used to plan routes for supervisory visits.
2. In addition to the map, a chart of local demographic information indicating the identified target population should be posted.
3. The office also needs either a large filing cabinet or a series of shelves for the filing system which requires: -
 - Manila folders- one for each peripheral health unit
 - Manila folders – one for each member of PHU staff
 - Files relating to correspondences
 - Files relating to logistics – stocks, inventories, requisitions, transport
 - Files to record the minutes of District Health Management Team meetings etc.;
 - Files for collection of the monthly records sheets;
 - Files relating to District Health Information
4. A small library containing books, manuscripts and documents related to Primary Health Care is recommended for used by the District Health Management Team members. The Library may contain:
 - Simple medical textbooks on health care
 - Textbooks on the management of Primary Health Care
 - Selected WHO publications;
 - Sierra Leone manuscripts and publications such as: -
 - The Country Resource Utilization Report
 - The National Action Plan for Primary Health Care
 - The Sierra Leone National Drug Formulary
 - Annual Report of the Ministry of Health and Sanitation
 - PHC Operational Handbook;
 - The Bamako Initiative Proposal
 - The up-dated Health manpower and health facilities list of the District.
 - The National Health Policy
 - The National Health Action Plan
 - District Health Profile report
 - District Health Action Plan etc.

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- National Eye Care Programme document.
 - Other Documents/Papers/research reports produced by MoHS or NGOs
5. Writing Equipment – Typewriter, stationery and office sundries. Computers, printer and other related equipment and software programmes.

The Functions of the District Health Management Team

The District Health Management Team should: -

1. Take responsibility for the planning, organizing and monitoring of the whole District Health Service including MCH clinics situated at District Headquarters.
2. Take responsibility for the planning, organizing and monitoring of the whole District Health Services.
3. Meet at regular intervals (Preferably monthly) and on an *ad hoc* basis to: -
 - Review the activities of the past month
 - Plan new activities
 - Solve problems and obstacles, which have arisen
 - Prepare a timetable for supervisory visits
 - Prepare and implement the training programmes
1. Organize the number and distribution of the Peripheral Health Units within the District to meet the requirements of population distribution and density i.e. to make PHC universally accessible (see Criteria for selecting PHU). Ref; point should be identified.
2. Revitalize the existing network of health units by providing equipment, drugs and supportive supervision.
3. Train staff of the PHUs in Primary Health Care activities and arrange regular workshops for this purpose.
4. Encourage Community Participation and development of Village/Area Development Committees.
5. Ensure surveillance and prompt notification of all epidemic-prone diseases and other notifiable diseases and take prompt action to control the outbreaks of such diseases.
6. Plan, organise and monitor intervention strategies against other priority diseases
7. Collect, collate and analyse information relating to health and the health services within the District and use it to monitor, evaluate and plan.
8. Improve the capacity and services of the District hospital to enable it to cope with the increased referrals resulting from improved PHC activity in the District.
9. Be responsible for posting all categories of staff within the District
10. Coordinate the activities of all health-related NGOs in the District.

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